

Progress Notes

Signed Jun 23, 2026

Progress Notes by Michelle Backus, RRT at 06/23/26 1153

See initial note for full COPD education

PULMONOLOGIST:

Dr. Vardi

EVENTS SINCE LAST VISIT:

Patient was recently hospitalized and returned to the ED due to inability to care for herself. Patient will be going to a rehabilitation facility at discharge.

HOME MEDICATION REGIMEN:

Albuterol MDI- 2 puffs-Q6prn, Albuterol 2.5 mg-Q6prn, Duoneb-QID, Pulmicort- 0.5 mg-BID, Ohtuvayre-BID, Dupixent 300 mg- every 14 days
Prednisone- 10 mg daily alternating with 5 mg

Dr. Vardi is stopping Ohtuvayre due to chest tightness and not providing any benefit to the patient.

NEW EDUCATION TOPICS COVERED:

Reviewed the concepts previously discussed.

ASSESSMENT:

Patient is currently using 3 liters per nasal cannula with saturations of 97% at rest. She is noted to have diminished breath sounds with expiratory wheezes.

PATIENT QUESTIONS OR CONCERNS WITH PLAN OF CARE:

None

HOME OXYGEN THERAPY:

2 L

CPAP/BIPAP THERAPY:

Last seen on 5.1.26 at DH - Reprogrammed Aircurve 10 bilevel to S mode, 24/8, RAMP off, Very low trigger, very high cycle
Patient has good compliance with her Bipap device

SOCIAL OR EDUCATION BARRIERS TO COMPLETION OF PLAN OF CARE:

Patient was receptive to the education that was provided. She would benefit from

continued follow up.

Patient is followed by Palliative care and uses Morphine for symptom management.

Patient does not currently have a caregiver that is able to take care of her in the home.

Patient is unable to walk or stand even with assistance.

DISCHARGE NEEDS:

Patient would need to continue with her triple maintenance therapy. Dr. Vardi is stopping Ohtuvayre due to patient experiencing increased chest tightness and providing no obvious benefit to the patient. She would need to continue with total smoking cessation. She would need to continue to be compliant with her Bipap therapy. She was educated to call her physician at the first signs and symptoms of an exacerbation. Patient is followed by Palliative care as an outpatient. Patient would benefit from a rehabilitation stay at discharge. Patient would need additional outpatient support to be able to return home.

PATIENT CARE FOLLOW-UP PLANS:

She has an appointment 6/30/26 with her PCP. She has an appointment on 7/17/26 with Dr. Vardi. This appointment may need to be sooner if instructed by Dr. Vardi.

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